

Cognitive Behavioral Therapy (CBT)

Introduction

CBT was developed from the ideas of Albert Ellis, the founder of Rational Emotive Behaviour Therapy (REBT), Aaron Beck, the founder of Cognitive Therapy (CT), and Donald Meichenbaum's cognitive behavior therapy (CBT).

Cognitive behavior therapy is combines both cognitive and behavioral principles and methods in a short-term treatment approach.

The techniques and ideas it uses **involve identifying** distorted or erroneous thinking patterns **and learning new ways** of thinking **which correct the** distortions and provide different methods of "cognitive restructuring" and lead to a more balanced life.

Cognitive Behavioral Therapy (CBT) is a form of psychological treatment that **explores the links between** thoughts, emotions, and behaviors.

CBT is a goal-oriented, time-based, structured treatment that is **effective for a wide range of** mental disorders. It is the **most widely researched** psychotherapy and has a **strong evidence-based** framework that supports the effectiveness of the treatment.

CBT is focused on learning to alter your **thoughts (cognitions)** and your **actions (behaviors)**, which is why it is called cognitive-behavioral therapy.

The Basis of CBT

"The key idea behind CBT is that **what you think** and do affect **the way you feel**".

Cognitive behavioral therapy or CBT works on the **assumption** that **your thoughts /beliefs** influence your emotions and behavior and that **by identifying** and addressing problematic thoughts

you can help to change your behaviour and experiences for the better.

Cognitive behavioral therapy (CBT) is a type of psychotherapeutic treatment that **helps people learn how to** identify and change **destructive** or disturbing thought patterns that have a negative influence on behavior and emotions.

Cognitive behavioral therapy **focuses on changing the automatic negative thoughts that** can contribute to and worsen emotional difficulties, depression, and anxiety. These spontaneous negative thoughts have a harmful influence on mood. **Through CBT, these thoughts are** identified, challenged, and replaced with more objective, realistic thoughts.

History of CBT

The adoption of cognitive-behavioral therapy progressed slowly over time and was considered controversial during its development.

Dr. Albert Ellis pioneered behavior therapy in the **1950s** with his work on helping patients identify and challenge **irrational thoughts**.

In the **1960s Dr. Aaron T. Beck** developed the practice for **cognitive behavioral therapy**. His theories on **cognitive distortions** helped evolve CBT to what we know today.

Psychiatrist Aaron Beck was the first to practice cognitive behavioral therapy. Like most mental health professionals at the time, **Beck was a psychoanalysis practitioner**.

While practicing psychoanalysis, **Beck noticed the prevalence of internal dialogue in his clients** and **realized how strong the link between thoughts and feelings can be**. He altered the therapy he

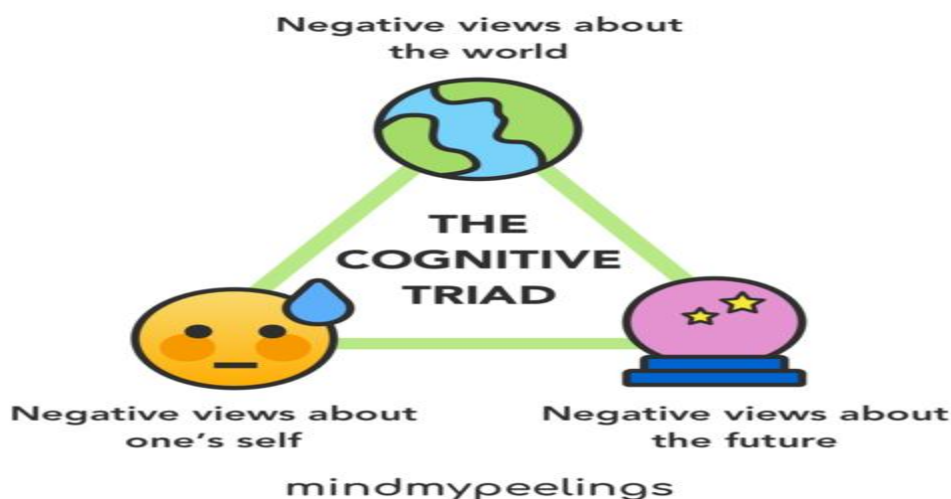
practiced in order to help his clients identify, understand, and deal with the automatic, emotion-filled thoughts that regularly arose in his clients.

Aaron Beck's approach to psychotherapy was groundbreaking and the scientific evidence today has proven the efficacy of his theories.

Dr. Judith S. Beck followed her father's footsteps and made a significant impact on CBT as well. She developed 10 principles of cognitive-behavioral therapy to provide an effective CBT treatment program.

Aaron Beck, defined three levels of cognition:

1. Core Beliefs
2. Dysfunctional Assumptions
3. Automatic Negative Thoughts



1. Core Beliefs

Core beliefs are learned and influenced by your early childhood experiences. They are deeply rooted in your belief system and define negative views you have about:

- **One's Self:** "I'm worthless and hate myself"

- **The World or Environment:** “Why does no one care about me”
- **The Future:** “Things will never get better”

2. Dysfunctional Assumptions (Cognitive Distortions)

Dysfunctional Assumptions occur because we tend to focus on the negatives. This causes a distorted perception of reality and misinterpretation of information.

These cognitive distortions are irrational thought patterns that are exaggerated by negative thinking and feelings.

Beck defined many common cognitive distortions that distort our perceptions of reality in a negative way.

3. Automatic Negative Thoughts

Automatic Negative Thoughts (ANTs) are involuntary negative perceptions of reality that occur habitually. They can be difficult to recognize because they are fleeting and cause negative emotions to occur.

You can challenge ANTs by altering your thoughts and reframing them more rationally and positively.

Beck's Cognitive Model

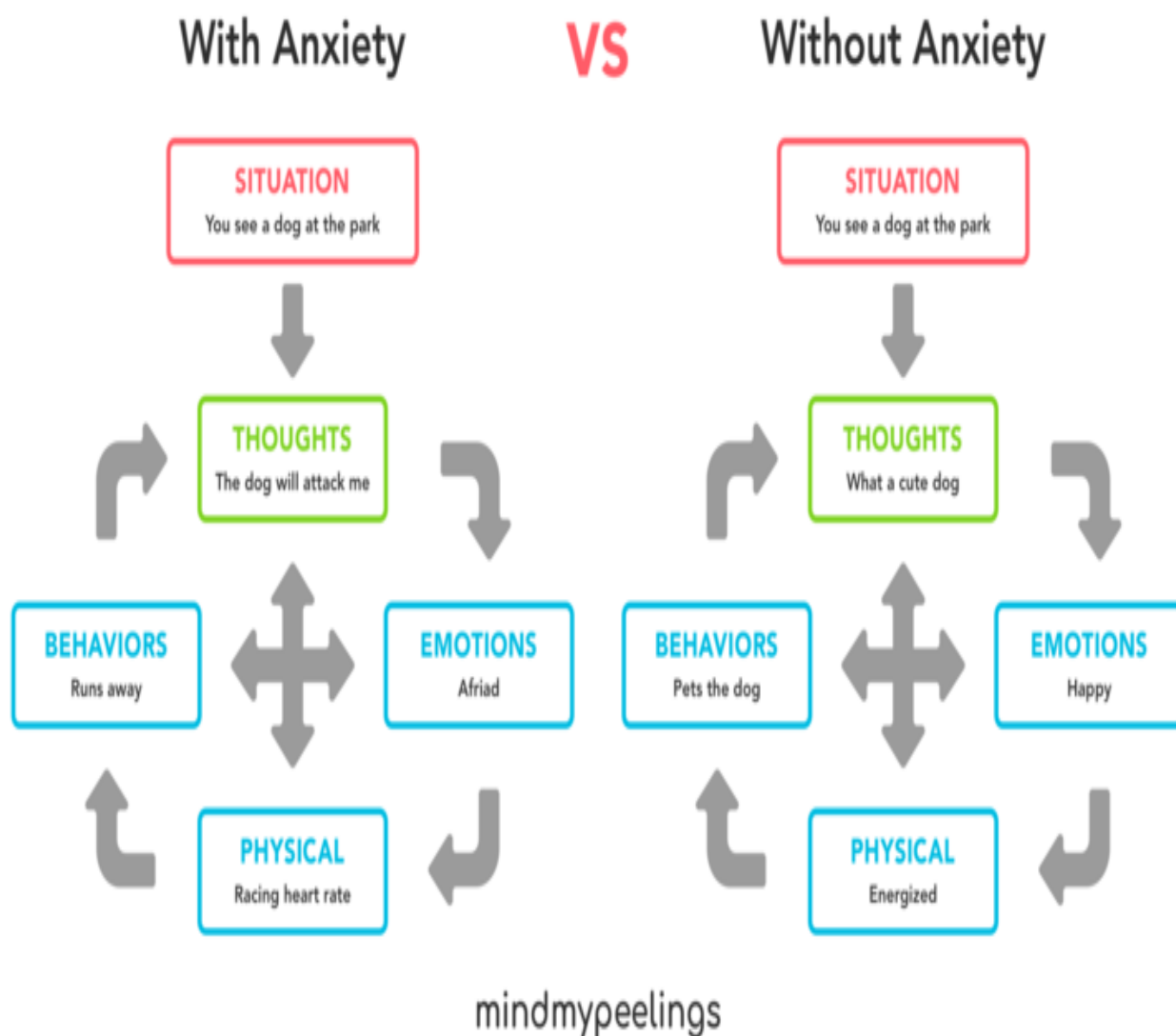


The CBT model was developed by Aaron Beck and used as a framework to understand a person's mental distress. This framework follows a simple process:

It starts with a distressing **situation/trigger** → which causes a person to have negative **thoughts** → this causes negative **emotions** and **physical distress** → which leads to negative **behaviors**.

Here is an example of someone who gets anxious around dogs compared to someone who isn't in the same situation:





This visual representation of your anxiety helps you understand what is actually happening. **It can help you realize that your thoughts are not facts** and seeing them laid out in this framework can help break the cycle.

CBT Treatment

Cognitive behavior therapy (CBT) is an evidence-based talking therapy that attempts cognitive and behavioral change based on an individualized formulation of a client's personal history, problems, and world views.

The key elements of CBT as **described by Beck included** engaging the patient, collaboratively developing a problem list, and deciding on a clear goal for the therapy session. Once the goal had been decided on, a CBT technique would be used (eg, guided discovery and Socratic questioning) to identify distortions in thinking style.

This would be followed by **an agreed task (homework)** for the patient to complete by themselves before the following appointment (eg, attempting to identify these distortions over the next week and trying to correct them).

Regular feedback and asking the patient to provide a capsule summary (ie, personal understanding) of the session were also crucial elements.

This therapy structure relied very much on collaborative working with the patient within an empirical methodology. **A formulation (narrative of the person's history) was jointly generated to make sense** of the emergence and maintenance of the problem at hand. This psychotherapy revolution placed the patient at the heart of the recovery process and introduced the concept of rational review of appraisal errors.

What You Will Learn in CBT

The main focus of CBT is to **alter your** negative thoughts and behaviors to be more rational. Throughout your CBT sessions you can expect to learn to:

- identify problems and build awareness of your negative thoughts and behavior

- recognize your thoughts are opinions and be able to distinguish between facts and irrational thoughts
- consciously challenge and reframe dysfunctional assumptions
- set achievable goals
- develop a more positive perspective of situations
- become your own therapist and practice relapse prevention

CBT can be effective for a variety of conditions, either alone or in combination with other therapies or medications. This includes:

- addictions
- anxiety disorders
- bipolar disorders
- chronic pain
- depression
- eating disorders
- obsessive-compulsive disorder (OCD)
- phobias
- post-traumatic stress disorder (PTSD)
- schizophrenia
- sexual disorders
- sleep disorders
- tinnitus

The bottom line

Cognitive behavioral therapy (CBT) is a well-established, effective type of short-term therapy. It's based on the connections between your thoughts, emotions, and behaviors, and how they can influence each other.

There are quite a few techniques that are used with CBT. Depending on the type of issue you want help with, your therapist

will help figure out which CBT strategy is best suited to your particular needs.

Types of Cognitive Behavioral Therapies

CBT encompasses a range of techniques and approaches that address thoughts, emotions, and behaviors. These can range from structured psychotherapies to self-help materials. There are a number of specific types of therapeutic approaches that involve CBT, including:

- **Cognitive therapy** centers on identifying and changing inaccurate or **distorted thinking patterns**, emotional responses, and behaviors.
- **Rational emotive behavior therapy (REBT)** involves identifying irrational beliefs, actively challenging these beliefs, and finally learning to recognize and change these thought patterns.
- **Dialectical behavior therapy (DBT)** addresses thoughts and behaviors while **incorporating strategies** such as emotional regulation and mindfulness.
- **Multimodal therapy** suggests that psychological issues must be **treated by addressing seven different but interconnected modalities**: behavior, affect, sensation, imagery, cognition, interpersonal factors, and drug/biological considerations.³

While each type of cognitive behavioral therapy takes a different approach, **all work to address the underlying thought patterns that** contribute to psychological distress.

Another view of Types ?

Dysfunctional Cognitions:

Should We Dispute, Question, Analyze, Accept, Validate, Ignore, Or Be Mindful of Them?

Conclusion

CBT systems, such as those developed by Beck or Ellis use direct cognitive interventions, sometimes called “disputation” or “cognitive restructuring.” But so-called third wave therapies -- DBT, Socratic questioning, ACT, mindfulness, behavioral activation, ERP -- and psychoanalysis as well -- all these therapies dispute dysfunctional cognitions indirectly and gently rather than directly and forcefully. So in attempting to change our patients’ dysfunctional cognitions, we therapists should continually ask ourselves, “Should we use behavioral or cognitive techniques.” And “Should we use disputation or indirect, gentle cognitive techniques, with this patient, with this dysfunctional cognition, at this point in treatment?”

Pros and Cons of CBT

Although the cognitive-behavioral approach has been proven to be effective for most people with a wide range of applications, it isn’t necessarily for everyone.

Here are the **advantages** of the CBT approach:

- can be completed in a relatively short period of time for most people
- can help treat some mental illnesses where medication alone has not improved symptoms
- focuses on altering your thoughts and behaviors to make changes to how you feel
- teaches you practical strategies that can be applied in your daily life
- provides the skills for you to be your own therapist enable you to be practical and prevent relapses

Some of the disadvantages of CBT are:

- requires the patient to play an active role and be committed to the process which can take a lot of effort and your time
- proves **difficult for people with more severe mental illness** or those with learning disabilities
- involves confronting your anxiety, this initial exposure can be uncomfortable for some people
- addresses the individual's needs, the **patient's environment (family and interactions) are not addressed** and can have a significant impact on their well-being
- the cognitive model focuses on a narrow scope and **focuses on only present problems** instead of underlying causes

Sayings on thoughts;

Epictetus

What disturbs men's minds is not events but their judgments on events.

Buddha

We are what we think.

All that we are arises with our thoughts. With our thoughts, we make our world.

What we are today comes from our thoughts of yesterday, and our present thoughts build our life of tomorrow: our life is the creation of our mind.

Shakespeare

Make not your thoughts your prisons.

There is nothing good or bad, but thinking makes it so.

► *"The stronger person is not the one making the most noise but the one who can quietly direct the conversation toward defining and solving problems."*

Another view of Types ?

Dysfunctional Cognitions:

Should We Dispute, Question, Analyze, Accept, Validate, Ignore, Or Be Mindful of Them?

By Milton Spett

With the exception of a few radical behaviorists, all cognitive-behaviorists **agree that dysfunctional cognitions are the core** component of psychological disorders. And nearly all cognitive-behaviorists **agree that changing dysfunctional cognitions** is the core component of effective psychotherapy. But **there is widespread disagreement within** CBT about *how* to change our patients' dysfunctional cognitions.

Dispute dysfunctional cognitions -- cognitive therapies

In the 1960s Albert Ellis proposed the then-revolutionary concept that psychological problems are caused by dysfunctional cognitions. (Ellis called them "**irrational beliefs.**") According to Ellis, the most important therapeutic intervention was "**disputation,**" teaching patients to dispute their dysfunctional cognitions and replace them with functional cognitions. (Ellis called the latter "rational beliefs.")

In the 1970s Aaron Beck argued that psychological problems are caused by **both dysfunctional cognitions and faulty information processing**. Beck viewed patients as **bad scientists, using** faulty information processing to develop and maintain erroneous theories (dysfunctional cognitions). Beck asserted that therapists should teach patients to become good, logical scientists. Beck used **logical arguments (disputation)** and behavioral experiments to change patients' dysfunctional cognitions.

Within a few years virtually all cognitive and behavioral therapists agreed with Beck that the essence of psychotherapy is changing

dysfunctional cognitions with logical arguments and behavioral experiments. And there the matter rested for a quarter century.

Third wave therapists maintain that cognitive interventions are unnecessary.

But then as the third millennium approached, a new wave of cognitive-behavioral therapies began to form. This movement has been called the **"third wave" by Stephen Hayes**. (Behavior therapy was the first wave and cognitive therapy was the second.) The therapies that formed this new, third wave agreed that **dysfunctional cognitions should not be disputed**. But the various third wave therapies have **presented a wide variety of alternatives** to disputing dysfunctional cognitions.

Validate dysfunctional cognitions -- dialectical behavior therapy (DBT)

The first third wave therapy actually has one leg in the second wave and one leg in the third. Marsha Linehan's DBT recommends both **validating and disputing** dysfunctional cognitions, but she places far **greater emphasis on validation**.

In DBT, **"validating" means** accepting as understandable or reasonable under the circumstances. For example, it is understandable that patients who have experienced traumatic events would develop dysfunctional cognitions about their safety.

The apparent conflict between validating and changing dysfunctional cognitions, emotions, and behavior is the core dialectic in DBT. According to Linehan, therapists must balance validation and change throughout treatment, in every session, and in every interaction with patients. Linehan points out that every intervention aimed at change can be experienced by the patient as invalidating.

The third wave focuses on behavioral interventions.

Linehan describes six types of validation:

1. Listening to, observing, and trying to understand what patients are saying, feeling, and doing.
2. Accurately reflecting back to patients the therapist's understanding of patients' expressed feelings, thoughts, assumptions, and behavior.
3. Articulating patients' unexpressed thoughts, feelings, wishes, etc.
4. Justifying patients' thoughts, feelings, and behavior in terms of past experiences or biological dysfunctions.
5. Justifying patients' thoughts, feelings, and behavior in terms of their current psychological state.
6. Articulating the patient's abilities and accomplishments; seeing patients as more than their psychological problems; viewing patients as individuals who are entitled to status and respect.
7. Expressing optimism that patients can overcome their problems, grow, and achieve more of what they want in life.

In Linehan's view, validation creates a strong therapeutic relationship, enabling the therapist to use mostly behavioral techniques plus some cognitive and paradoxical techniques to change dysfunctional cognitions.

Question dysfunctional cognitions -- many therapies

Many cognitive-behavioral therapists recommend "Socratic questioning" to change dysfunctional cognitions. Socratic questioning means asking questions which will gently lead patients to change their dysfunctional cognitions without directly disputing those cognitions. Some of the therapists who recommend this approach are David Barlow in his Unified Protocol for Emotional Disorders; Patricia Resick in her Cognitive

Processing Therapy for PTSD; David A. Clark's CBT for obsessions; and Craske & Barlow's Panic Control Treatment.

Resick et al. (In Barlow, ed. *Clinical Handbook of Psychological Disorders*, 2008, p. 97) give an example of Socratic questioning. The patient had developed PTSD after shooting at a car that ignored warnings to slow down as it approached a checkpoint in Iraq. The car contained an innocent family, and several family members died. First the patient wrote a complete description of the events, his behavior, and his feelings. Next the therapist encouraged the patient to read his description and fully experience his emotions.

Finally the Socratic questioning:

Therapist: If a person shot someone but didn't intend to do that, what would we call that?

Patient: An accident, I guess.

Therapist: That's right, in fact, what would you call shooting a person when you are trying to protect something or someone?

Patient: Self-defense.

Note that by asking questions, the therapist is clearly suggesting that the patient change his view of himself and his actions.

Accept dysfunctional cognitions -- acceptance and commitment therapy

Acceptance and commitment therapy (ACT) asserts that the avoidance of cognitions and emotions is a key cause of psychopathology. A critical component of ACT is asking patients to accept and experience their cognitions and emotions, without judging or trying to change them. ACT uses mindfulness and imagery exercises to help patients experience rather than avoid cognitions, emotions, and memories. ACT also encourages

patients to pursue their valued goals in spite of any conflicting cognitions and emotions.

ACT asserts that cognitive restructuring (disputation) adds nothing to behavioral techniques. In fact cognitive restructuring violates ACT's basic assumption that dysfunctional cognitions should be accepted rather than judged or disputed. ACT asserts that the more patients attempt to suppress their dysfunctional cognitions and emotions, the more intense those cognitions and emotions become.

ACT also asserts that when patients stop trying to suppress dysfunctional cognitions and emotions, those cognitions and emotions will diminish in intensity, making it easier for patients to pursue those life goals they value most. But note that asking patients to pursue the life goals they value most, in spite of conflicting cognitions, clearly implies that those cognitions are dysfunctional or at least irrelevant.

Notice dysfunctional cognitions -- mindfulness-based cognitive therapy (MBCT)

In MBCT dysfunctional cognitions are simply "noticed." Instead of focusing on the content of negative cognitions, MBCT teaches patients to view these cognitions as events that pass through the mind, and to non-judgmentally observe this process as it occurs. "Decentering," refers to two basic concepts in MBCT:

1. "My thoughts are not facts" and
2. "My thoughts are not the center of my being."

When patients become aware of a dysfunctional cognition (or emotion), MBCT instructs patients to:

1. Observe your cognition without evaluating, planning, or acting on it.
2. Focus on your breathing for a minute or two.
3. Expand your attention to your whole body for a minute or two.

4. Notice how the cognition (or emotion) affects your bodily sensations.

MBCT does not directly dispute dysfunctional cognitions, but by instructing patients not to act on these cognitions, and not to assume that these cognitions are accurate, MBCT certainly *implies* that dysfunctional cognitions are inaccurate.

Ignore dysfunctional cognitions I -- behavioral activation

Behavioral activation (BA) focuses on the consequences of behavior. Behavior that helps patients achieve their goals is encouraged, while behavior that prevents patients from achieving their goals is discouraged. For example, suppose a patient wants friends, but believes that everyone hates him. A cognitive therapist would try to convince the patient that this belief is incorrect. But a BA therapist would ask the patient, "What are the consequences of this belief?"

The consequences of believing everyone hates you are avoiding people and not having friends. But one of the patient's goals is to have friends, and patients recognize that avoiding people prevents them from having friends. So the BA therapist encourages patients to interact with people in order to achieve their goal of having friends.

BA theory also asserts that patients must keep struggling with their old avoidance habits until they are replaced by new active habits. BA therapists do not dispute dysfunctional cognitions, but by asking patients to disregard their dysfunctional cognitions, BA conveys the impression that these cognitions are unhelpful or incorrect.

Ignore dysfunctional cognitions II -- exposure & ritual prevention

Edna Foa recommends exposure and ritual prevention (ERP, sometimes called "exposure and response prevention") for

several anxiety disorders, most notably OCD. Foa believes that the core problem in OCD is dysfunctional cognitions about danger. But her treatment pretty much ignores these cognitions and focuses on behavioral interventions -- exposure and ritual prevention.

OCD theory asserts that obsessions create anxiety, and compulsions are rituals designed to reduce this anxiety. Exposure means experiencing the obsession or experiencing the situation that provokes the obsession and the urge to perform a ritual. Ritual prevention means not performing that ritual. Foa asserts that frequent and prolonged ERP will change the OCD patient's dysfunctional cognitions regarding danger.

For example, if the obsession is the thought "What if I have AIDS?", the ritual might be getting tested for AIDS. When the test results are negative, the patient feels better for a while, but after a few days or weeks the patient will begin to wonder "What if the lab made a mistake? I better get tested again to be sure." And this cycle of obsession, ritual, and temporary anxiety reduction will continue on and on and on. According to Foa, the best way to change the patient's dysfunctional cognition that he may have AIDS is to experience the obsessive fear, and just wait for the fear to run its course without performing any compensatory behavior designed to reduce the fear.

Foa's theory is that exposure works because it provides evidence which disconfirms the patient's dysfunctional cognitions about danger. Foa does not recommend that therapists dispute, question, validate, accept, or use mindfulness techniques to change patients' dysfunctional cognitions. But telling a patient who believes he has AIDS to refrain from getting tested clearly suggests that this patient does not have AIDS.

Analyze dysfunctional cognitions: Psychoanalytic therapy

The essence of psychoanalytic therapy is *analysis*, helping patients identify the putative underlying causes of their dysfunctional cognitions, emotions, and behavior. These underlying causes can be either in the patient's childhood, or in their current psychological structure. Analytic theory asserts that an empathic therapeutic relationship will help patients to experience and understand their inner lives (make their unconscious conscious) and then dysfunctional cognitions, emotions, and behavior will remit.

In a workshop I once gave with a psychoanalytic therapist, I described how I would treat several hypothetical cases, and the psychoanalytic therapist described how she would treat those same cases. The basic difference between us was that I would say to patients "You should go home and do X," while she would say "I wonder why you don't go home and do X."

Note that analysts do not ask their patients to analyze every cognition, emotion, or behavior. Analysts only ask patients to analyze those cognitions, emotions and behaviors that the analyst judges to be dysfunctional. Telling patients you wonder why they don't do X gently implies that they should do X, and that their cognitions justifying their not doing X are dysfunctional.